

The Level of Mental Health Literacy of Filipino Adults in Metro Manila at the Time of COVID-19 Pandemic

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Abstract

The outbreak of COVID-19 has caused the increase of reported mental health problems in the Philippines while the country struggles with mitigating the detrimental impacts of the corona virus disease 2019 pandemic. Filipino adults became one of the most vulnerable to experience the pandemic's negative psychological impact. Hence, concerns suddenly become a subject of discussion among the public in acknowledging the importance of mental health literacy. The researchers conducted this study to determine the level of mental health literacy (MHL) of Filipino adults amidst the COVID-19 pandemic. The instrument by O'Connor and Casey: Mental Health Literacy Scale (MHLS) was used to survey 130 respondents residing in Metro Manila. The results show that the respondents got a moderate MHL level (2.74). Among the six mental health literacy components, knowledge of self-help intervention (3.15), attitudes of recognition and help-seeking behavior (3.12), mental illness recognition (3.11), and knowledge and beliefs about the support available (2.96) were found at a moderate level while knowledge about risk factors and causes (2.24) and knowledge of mental health influences (2.45) were in marginal level. Socioeconomic status was a significant factor in the knowledge of the self-help intervention ($p=0.044, \leq 0.05$), knowledge, and beliefs about the support available ($p=0.008, \leq 0.05$), attitudes of recognition and help-seeking behavior ($p=0.007, \leq 0.05$), and knowledge about risk factors and causes ($p=0.006, \leq 0.05$). The highest educational attainment was significant to mental illness recognition ($p=0.015, \leq 0.05$), knowledge and beliefs about the support available ($p=0.015, \leq 0.05$), and knowledge of mental health influences ($p=0.009, \leq 0.05$).

Keywords: Mental Health Literacy (MHL), COVID-19 Pandemic, Filipino Adults, Help-seeking

Introduction

On December 12, 2019, an unknown virus with viral pneumonia-like symptoms was detected in the Huanan Seafood Wholesale Market in Wuhan, China, where the existence of the virus is believed to have emerged (Handley, 2020). Accordingly, based on a timeline of events of the widespread mysterious, lethal virus after the following days, the World Health Organization identified the virus as a new coronavirus variant and was named 2019-nCoV or COVID-19 (the 2019 novel coronavirus) (Handley, 2020). The World Health Organization (WHO) immediately announced the onset of the COVID-19 virus on March 11 as a global pandemic and health emergency due to its rapid spread and alarming health impacts. The virus caused an exponential surge of cases recorded in more than 100 countries by abruptly putting everyone's physical health safety needs on the line and jeopardizing the probable socio-economic collapse (MalindogUy, 2020). This is evident especially amongst developing countries, including the Philippines, which became one of the most vulnerable countries to the virus's detrimental impacts. It made the Philippines prone to the threat posed by the virus; hence, President Duterte promptly placed the whole

country in a "state of calamity" on March 16 (Malindog-Uy, 2020). The country's battle towards mitigating the spread of the COVID-19 virus and its impacts posed an intolerable challenge in proving its preparedness to maximize its resources by utilizing the total capacity of healthcare infrastructures, workforce, and the usage of medical supplies and equipment, as well as efforts in imposing several safety health protocols and precautions directed to save Filipinos' lives and mitigate the pandemic impacts. The Philippine National Capital Region (NCR), including Metro Manila, was reported to become the epicenter of the COVID-19 virus. According to the statement of Health Undersecretary Maria Rosario Vergeire, 2,000 average daily COVID-19 cases are added to the officially recorded tally of COVID-19 cases in the Philippines. Based on the Department of Health (DOH), the Philippines has a nationwide record tally of COVID-19 cases at 2,800,621 as of November 6, 2021, according to the article report of GMA news of Noriega, 2021. With regards to this, a whopping number of COVID-19, public officials, and local government units (LGUs') were prompted in mandating actions toward safeguarding the curve of the total cases of COVID-19 and preventing its spread. Thus, several health safety preventive measures are introduced, such as doing basic social distancing,

wearing facemask and face-shield, and mandating temperature-check and hand disinfection in every local and private establishment. Coupled with these health precautions is the government's implementation of heightened health precautions and safety protocols, such as the imposition of different versions of community lockdowns, quarantine guidelines, curfew hours, and eventually the urgent vaccination programs. While these efforts and actions are intended to sustain the Filipinos' physical safety needs from getting infected with the virus, unforeseeable consequences on mental health concerns take a toll. The general public's sudden priorities of physical health safety needs have shifted over mental health needs.

The Department of Health (DOH) reported at least 3.6 million Filipinos suffering from mental health problems as the Philippines continue to struggle with the COVID-19 pandemic. In a report written by Domingo (2021), in reference to Regional Resource Mapping 2019-2020 conducted by the Department of Health Philippines, depressive disorder is the most prevalent disorder with a rate of 1.10%, followed by alcohol use disorders with a 0.90% prevalence, drug use disorders with 0.70%, epilepsy with 0.30%, and bipolar disorder and schizophrenia with 0.20%. Moreover, it was also reported that there is a 5.4% rate of suicide deaths per year. Additionally, among the 1,879 Filipinos, specifically, adults surveyed, 25% said they have moderate to severe anxiety, and 16.67% said they have moderate to severe depression and psychological impact. Filipinos who's in their adulthood become vulnerable have a higher risk of being infected with the coronavirus as postulated by the Department of Health (2020); World Health Organization, 2020; and Buenaventura R., Ho J. B., &Lapid M. I. (2020). While based on Domingo (2021), the Philippine Statistics Authority stated that in 2020 the death due to suicide rises to 3,529 (25.7%), which is higher compared to 2019, that of 2,808 deaths due to self-harm.

Mental health has been defined as a subjective state of well-being in which one realizes his/her abilities and has the capability to cope with day-to-day struggles of life and can work productively, as well as have a significant contribution to a community (WHO, 2014 p.1). The COVID-19 pandemic impedes the very core of this definition of mental health. By disrupting the lives of Filipinos in meeting their social, physical, and mental health needs by restricting them needs to socialize and do their regular outdoor activities such as attending work regularly in the office, attending a mass, as well as attending outdoor family gatherings and alike were temporarily prohibited and became

limited. Moreover, according to the report of Avasthi and Grover in 2018, these simple impediments effects brought by abnormal situations like this pandemic can bring an experience of collective stress and anxiety as well as overwhelming emotions to everyone that can potentially impede and threaten one's psychological health aside from safety survival. Moreover, according to the report of Avasthi & Grover (2018), it is not easy to cope with prolonged confinement, and amidst the mandated isolation, it was further added that one might experience various mental health syndromes that could lead to mental health disorders (s), if not properly recognized by someone such as depressive symptoms, loneliness, pessimism, deteriorations in cognition, and disruption in sleeping patterns. These are aligned with the known psychological reactions of stress, anxiety, loneliness, and agitation brought by this pandemic, as inferred by Meng et al., 2020.

Adults' spiritual needs have been affected due to restrictions in attending church services and restrictions on religious practices, which has increased their feelings of isolation, sadness, and longing and aggravated their feelings of uncertainty. This also leads to poor social well-being as adults who have no moral support at home suffer much more significant difficulties. The majority have very little or no knowledge of technology, which further isolates them. This lack of social engagement induces long-term distress and negatively impacts their emotional well-being (Newman and Zainal, 2020). Furthermore, prolonged lockdowns negatively impact adults' physical well-being, resulting in a minimization of their physical activity, leading to a more sedentary lifestyle. Hence, even the National Center for Mental Health (NCMH) acknowledges that there have been an increasing number of suicide-related calls in its crisis hotline since the pandemic's start. The majority of those who called are reported from Filipinos residing in Metro Manila, the epicenter of the outbreak (Huaxia, 2020).

However, in the Philippines, most people with mental issues and illness are often faced stigmatized and called derogatory names such as *sintu-sintu*, *papansin*, *may tililing*, *baliw*, *taong-grasa*, regardless of age differences, which entails the simplistic views and attitudes of Filipinos towards mental health (Rivera & Antonio, 2017), that further calls for mental health education among adults to an older generation before us. One of the common misconceptions and often stigmatized among Filipinos is that depression and anxiety don't exist and that mental illnesses are something to be ashamed of. Tanaka et al. (2018) found that this stigma results from public perceptions

of mental disorders, which are divided into three categories. The first is familial problems, in which a family member with a mental illness is rejected or disowned because they feel that it can be hereditary. Second, there is a false pessimism and optimism about the severity of the illness, implying that the mentally ill will either suffer significant functional damage or be able to cope with any psychological anguish on their own. Finally, there's the oversimplified chronic course, in which people who don't have mental illnesses apply an acute illness model to people who are sick and expect a quick recovery. Likewise, according to the systematic review of Martinez, Co, Lau & Brown in 2020, they pointed out several barriers that influence Filipinos' help-seeking behavior and might contribute to Filipinos' awareness and poor mental health literacy when it comes to various mental health components. These include the following factors: financial constraints, self-stigma, sense of shame, fear of being labeled as 'crazy,' self-blame, resiliency, conformity to norms, and concern for the loss of face. When it comes to risk factors and causes of mental illness components, a series of studies show that many traditional practices and cultural beliefs still exist and are prevalent among Filipinos due to Filipinos' different lay conceptualization practices and beliefs of root causes of psychological disorders that differ from the medical model (Araneta, 1993; Tan, 1987, 2008). These traditional practices and cultural beliefs include attribution of mental illnesses to spiritual and supernatural causes—God retribution, spirit possession, witchcraft (*mangkukulam*), Edman and Kameoka, 1997; soul loss, lack of balance, nightmares (*bangungot*), and pollution/contagion (Araneta, 1993; Tan, 2008; preference for an initial consultation on indigenous or folk healers for 'disturbing behavior,' as well as for somatic complaints that have no verified underlying medical causes, prevalent among inhabitants in rural areas, Shakman (1969). In general, all of these cultural beliefs and practices play a crucial role in shaping Filipino's opinions, decisions, values, and actions, consequently influencing their choice of self-help interventions and help-seeking behavior.

It is evident enough the susceptibility of Filipinos to psychopathology and psychological distress, worsened by the COVID-19 pandemic. Several movements such as the Mental Health Act of 2017 and the Mental Health program have recently been implemented and passed. In line with this, the first mental health act legislation in the Philippines' history was passed into law, known as the Republic Act no. 11036, on June 21, 2018. It provides a rights-based mental health bill and a comprehensive framework for implementing optimal

mental healthcare in the Philippines to all Filipinos (Lally, Samaniego, Tully, 2019). And one of its provisions mandated the right to be adequately informed about psychosocial and clinical assessments... to aid their future actions to mental health conditions. Hence, the present study was supported by this law that further establishes its significance in studying the mental health literacy (MHL) of Filipino adults in the Philippines.

Yet, despite this advancement in the area of legislation of Mental Health in the Philippines, the mental health literacy of Filipinos, especially the adult ones, still become left under-investigated and given less attention in the field of research. Although a volumetric review of literature on how traditional Filipino values and cultural paradigms influence Filipinos' conceptualization of mental health literacy are available, little is known about how modern-day adult Filipinos perceive the various components of mental literacy as proposed by Jorm et al. (1997). Along with these, a call for a need for further investigation of Mental Health Literacy (MHL) in the Philippine settings must be made. Recent studies have focused only on mental health itself, and there are only a few studies about mental health literacy on how it is associated with Filipinos. Studies of mental health have not adequately examined mental health literacy in the Philippines. There are also insufficient numbers in the published literature that examines the Mental Health Literacy of Filipino adults, specifically 40 to 65 years old. Most studies on Mental Health Literacy in the Philippines settings were usually done only center among teenagers and young adults' population with age ranges from ages 13-35 years old. Mental health literacy (MHL) is known as knowledge and beliefs about mental disorders, which aid their recognition, management, or prevention (Jorm et al., 1997, p. 182). Jorm and his colleagues proposed that it comprises of six components as follows: (a) mental illness recognition; (b) knowledge of self-help intervention; (c) knowledge and beliefs about the support available; (d) attitudes of recognition and help-seeking behavior; (e) knowledge about risk factors and causes (f) knowledge of mental health influences. These six components determine a person's overall mental health literacy that was examined in the present study among Filipino adults.

The present study aimed to determine the current levels of mental health literacy of Filipino adults residing in Metro Manila during the time of the COVID-19 pandemic using a scale of Connor and Casey (2015), The Mental Health Literacy scale (MHLS). The scale encompasses the above mentioned

six determined mental health literacy components. It also focused on the mental health literacy levels of the target respondents and their ' demographics, as well as its implications as a basis for designing a mental health psychoeducational program (s) dedicated to raising awareness and enhancing the mental health literacy of Filipino adults residing in Metro Manila.

As a student, the researchers were inspired to conduct the study for the following reasons. To determine the level of mental health literacy (MHL) of Filipino adults who are least studied in mental health topics amidst the peak of the COVID-19 pandemic. And aligned to the researchers' personal experiences observed in their older family household members, they commonly hold a misconception and false attribution of mental illnesses on traditional beliefs and opinions. Hence, posting sparks their interest in delving into this, aside from this, to broaden further the researchers' knowledge of the mental health literacy of Filipino adults.

Research Questions

This study attempted to determine the Mental Health Literacy (MHL) level of Filipino adults living in Metro Manila during the COVID-19 pandemic. The following questions were addressed in this study:

1. What is the general level of Mental Health Literacy (MHL) among Filipino adults in terms of:
 - 1.1 Mental Health Recognition;
 - 1.2 Knowledge and beliefs about self-help interventions;
 - 1.3 Knowledge and beliefs about the support available;
 - 1.4 Attitudes that facilitate recognition and appropriate help-seeking and;
 - 1.5 Knowledge and risk factors and causes and;
 - 1.6 Knowledge of mental health influences?
2. Is there a significant difference between the demographic profile of the respondents and the six mental health literacy (MHL) components when grouped according to:
 - 2.1 Socioeconomic Status;
 - 2.2 Religion;
 - 2.3 Marital Status and;
 - 2.4 Highest educational Attainment?

Literature Review

The Six Components of Mental Health Literacy

In 1977, Jorm et al., introduced and pioneered the

study on mental health literacy; they defined the construct as: Knowledge and beliefs about mental disorders that aid an individual recognition, prevention, and management towards mental health.

Accordingly, O'Connor and Casey (2015) expounded the two components by Jorm into six determined mental health components. First is the ability to recognize specific disorders in which a person has the ability to correctly identify features of a disorder, a specific disorder. Second is the knowledge and risk factor and causes; this means that a person has an awareness of the cause and risk of developing a mental illness. The third is the knowledge of self-treatment that signifies a person's knowledge of treatments recommended by mental health professionals. Fourth is the knowledge of professional help available this denotes the person's knowledge of mental health professionals and the services they provide. Fifth is the knowledge of how to seek mental health information; this indicates a person's awareness of how and where to access information related to mental health. Lastly, attitudes that promote recognition and appropriate help-seeking imply the attitudes that impact the recognition of disorders and willingness to engage in help-seeking behavior.

These components are determinants of people's outlook and actions in managing their experiences towards their psychological disorders or distress, concerning their mental health that is influenced further by one perceived mental health literacy. And frequently, under the Philippine setting, this issue still calls for research examination, despite the country long history of narrative progression in psychiatry and mental health progression during its history of colonizers period namely, the following: the pre-Spanish and Spanish era, the American regime, the Japanese occupation, as well as American liberation, (Samiengo, 2017). Yet, despite all of these revolutionary periods, this present 21st century wherein mental illnesses are gradually viewed and accepted through the lenses of medical science.

On Mental Health Recognition

In a study conducted by Furnham and Hadjimina in 2017 on the influence of age and gender on the mental health literacy of anxiety disorder on one hundred sixty-two European participants. Participants were divided into three groups, 18- to 29-year-olds that fell at the young age group, 30- to 44-year-olds fell in the adulthood group, and 45- to 71-year-olds fell into the middle age group. Upon examining general anxiety, the respondents did consider it a significant problem,

influencing recognition rates and reasonable treatment beliefs, leading to higher ratings of recognition of the Filipino adult respondents. Adding to this probably perceived recognition of the respondents is the presence of mass media representation of mental health. A study done by Ghuloum et al. (2010) examined the knowledge, attitudes, and practices concerning mental illness among Qatari and other Arab expatriates. They postulated that the most common information source on mental health awareness was the media, with 64.2% dependency on recognizing common mental disorders in the studied population. Furthermore, this aligns with the study of Furham in 2012 in investigating the Psychiatric literacy and the personality disorders of lay people. It was stated there that the more lay people are exposed to specific terms such as 'paranoid' and 'obsessive-compulsive', that are frequently represented in the media. For instance, the term 'paranoid' is often used in relation to discussions regarding theories in the media, documentaries, and lyrics of songs. Other mental health conditions are also portrayed in film and news that could potentially contribute to people's awareness in recognition of mental illness.

Knowledge of Self-help intervention

A study about the social and cultural context of the mental health of Filipinas in Queensland found that Filipinos have a high tolerance for dealing with their mental health struggles on their own and that they value being self-sufficient in coping. They believe that their mental health is just a temporary state,' and that the problems that occur from it are just as temporary and situationally dependent, hence it does not require professional help (Thompson S, Manderson L, Woelz-Stirling N, Cahill A, & Kelaher M., 2002). Additionally, to restore, massage (similar to acupressure or reflexology) and 'magnetic healing' (where the healer's hands are placed on the damaged area while praying or meditating) are utilized. The flow and balance of life force in its natural state (Araneta, 1993; Tan, 2008). Herbal medications and massage are used by bonesetters (manghihilot) and arbolarios to treat pain, reduce anxiety, and improve the mental condition (Araneta, 1993; Lagaya, 2005). These are some of the Filipinos' folk healing techniques, and present traditional and complementary medicine is observed to be a combination of indigenous methods dating back to Spanish colonization, as well as influences from Ayurvedic and Chinese traditional medicine (Lagaya, 2005).

Knowledge and Beliefs about the Support Available

On a review of the literature of mental help-seeking among Filipinos written by Tuliao (2014), it was emphasized that in the Philippines, the mental health services are underutilization--lack of professionals and services provided by mental health professionals that are legally recognized in the country this includes guidance and counseling professionals (Guidance and Counseling Act of 2004); psychologists (Philippine Psychology Act of 2009); social workers (Republic Act No. 4373 (1965)); and those within the scope of these medical professions (Tuliao, 2014). Currently, with the utilization of media access and equipping oneself on the knowledge and beliefs about the support availability that mental health professionals provide become convenient to everyone with just a go search on our fingertip with the rise of technological advancement that pave the way for telephone counseling and online counseling. According to the study of Hechonova's et al., In 2013, in investigating the factors that influence facilitating seeking among overseas Filipino workers (OFWs), it was revealed that three factors could determine help-seeking behavior among OFW Filipinos in using online counseling. These include technology adoption, perceived ease in using technology, and perceived presence of the organization and technological infrastructure to support use, which further entails that exposure to this kind of treatment might add up to the awareness of the respondents on the availability of the support and service provided by mental health professionals.

Attitudes of recognition and Help-Seeking Behavior

In the study of Gong, Gage, and Tacata (2003), they conducted interviews with Filipino Americans who are Bilingual and live in San Francisco, California, or Honolulu, Hawaii. They also split mental health care systems into four groups (lay, mental health specialty, general practitioner, and folk systems) to better understand Filipino Americans' help-seeking behavior. In comparison to the professional and folk domains, the lay system (asking a friend or relative for aid) is the most common source of care for Filipino Americans. Additionally, Straiton, Ledesma, and Donnelly (2018) studied 14 Filipino informants (particularly Filipinas) between the ages of 18 and 49 who lived in Norwegian for one to six years. According to the findings, Filipino women generally view their mental health issues as temporary and situational, and hence do not require professional help. The women's willingness to seek professional aid did not appear to be related to their level of emotional distress. Furthermore, the majority of Filipina informants who had suffered a mental health problem drew strength from their strong support networks and

their religious beliefs to help them get through their troubles. Furthermore, twenty-five of the thirty women migrant live-in caregivers studied by Vahabi, and Wong (2017) were Filipina. According to the results, many migrant caregivers have limited access to social support.

The majority of participants said their primary sources of social support were their family back home, new acquaintances in Canada, and church groups. They also recognized trust as a crucial element in deciding who they would seek help from or believe in. Furthermore, Professional help is needed primarily in the absence of social support; therefore, the availability of sympathetic, reliable, and trusted family and friends is harmful to formal help-seeking. This is due to the prevailing cultural beliefs that regulate Filipino interpersonal connections, known as *kapwa* (shared identity), in which trustworthy family and friends are regarded as "hindi-ibang-tao" (one-of-us/insider), whereas doctors and professionals are regarded as "ibang-tao" (outsider). Filipinos are more likely to disclose and be open and honest about their mental illness to individuals they consider to be "hindi-ibang-tao" (insiders) rather than "ibang-tao" (outsiders), resulting in a preference for family and close friends as informal aid sources. It is difficult for Filipinos to trust a mental health practitioner who is not a family member.

Knowledge of risk factors and causes

In the Philippines, people with mental illness were widely transpired because of the power of the media to influence and shape one's beliefs. The media usually portrays people with mental illness as someone dangerous and treated comically. According to Tuliao (2009), people who are suffering from substance addiction were portrayed as criminals, murderers, rapists, and sexual deviants in Filipino tabloids and broadsheets. Hence, it was also found that stigma continues to be the most significant impediment to the development of the mental health system in the Philippines. In the study of Thompson et al. (2002), the conceptualization of Filipinos on causes of mental illness was emphasized on personal responsibility. Severe mental problems are believed to be caused by the softness of character and specific traits. Thus, the ability to deal with one's emotional concerns is valued and also considered as a personal responsibility (Thompson et al., 2002). Additionally, some psychological and physical illnesses are said to be caused or caused by evil activities. Tan (2008) also discusses various Filipino beliefs that physical and psychological problems are caused by *sumpa* (curse)

brought on by a violation of traditional family values, or by *gaba*, a divine curse or punishment. Finally, when it comes to recognizing dissociative disorders, Filipinos were more likely to define symptoms as a result of spirit possession than as a result of a psychiatric condition (Gingrich, 2006). Moreover, it was found in the study of Picco et.al 2019 that stigma is thought to be the result of a combination of three issues: a lack of understanding (ignorance), bad attitudes (prejudice), and exclusion or avoidance actions (discrimination).

Knowledge on mental health influences

In 2008, the World Health Organization stated that stigma is also considered one of the significant determinants of social exclusion. In the recent systematic review of the relationship between mental health and social exclusion, they identified that inadequate knowledge on mental health can result in discrimination against the people suffering or who have a mental illness. It is rampant in every aspect like economic, political, and social exclusion (Curran et al., 2007). In the American Psychiatric Association (APA) article, people with mental illness are marginalized and discriminated against. They emphasized the three types of stigmas which are public stigma, self-stigma, and institutional stigma. In public stigma, it refers to the negative or discriminatory attitude of others towards mental illness. In self-stigma involves the negative attitude that a person with a mental illness has about their condition, including internal shame. Lastly, the institutional stigma, where they labeled it as more systematic because it involves an involvement of policies both government and private organizations that intentionally or unintentionally limit opportunities for people with mental illness (American Psychiatric Association, 2021). Additionally, in a study conducted by Tanaka's et al. in 2018, they documented the rampant stigmatization experienced by Filipinos with mental health problems with epilepsy conditions in the Philippines. It highlighted the factor of culturally and socio-economically specific situations that could result in influential modifiers of the stigmatized experience. In their study, participants faced a stigma due to cultural characteristics such as the perception of mental health problems as a family illness.

COVID-19 Impacts on the Mental Health of Filipino Adults

The Philippines has the highest number of coronavirus infections, with a mortality of 40% (Lema & Morales, 2020). The National Center for Mental Health (NCMH) stated that there had been an increasing

number of suicide-related calls in its crisis hotline since the start of the pandemic. Major calls they received were residing at Metro Manila as the outbreak's epicenter. The average monthly calls after the pandemic's onset more than doubled compared to the pre-pandemic months. Based on the Department of Health (DOH), they partnered with The World Health Organization (WHO) in raising public mental health awareness since this pandemic has been difficult for the Filipinos (Xinhua, 2020). The onset of the COVID-19 pandemic caused an alarming spike of calls by many seeking help, as cited by the National Center for Mental Health (NCMH). It was reported that they received an average call of thirty-two (32) up to thirty-seven (37) calls in a single day starting from March 17 to October six. Moreover, throughout this pandemic, it was found out that there were three top reasons for the alarming numbers of calls. It was stated as follows: anxiety-related concerns, asking for a referral to a psychiatrist, and inquiry regarding hospital services (Esguerra, 2020). Yet, despite this growing awareness of mental health and the recent approval of the National Mental Health Law (RA 11036) in the country, the Department of Health (DOH) still acknowledges the need to address the mental health problems during the national crisis.

Another factor is the unmet spiritual needs of Filipinos. The majority of Filipinos are deeply religious and have strong views of God's role in health and well-being. According to Buenaventura et al. (2020), Filipino adults were unable to participate in church services every Sunday because of the COVID-19 pandemic. This impacted their mental health since they lost their association with their spiritual leaders and church peers. Filipinos' religiosity and deep spirituality serve as a strength and protective buffer to stress and suffering (Esteban, 2015). In addition to that, Philippine culture is known for the concept of multigenerational households, where multiple generations of families, including grandparents, parents, children, and grandchildren, live under one roof, so the thought of dying alone is one of the impacts of COVID-19 outbreak on the mental health of Filipino people. The majority of respondents spent an average of 0-9 hours on social media to gather information and news during the initial period of the lockdowns in the Philippines. Many who had access to current knowledge and were pleased with it had fewer psychological impacts and lower stress, anxiety, and depression levels. Many people still feel the need for more up-to-date details. They displayed anxiety and had a mild psychological impacts, contributing to "headline stress disorder" (Dong and Zeng, 2020). The study's key hypothesis is that the COVID-19 pandemic

predicts stress, depression, and anxiety in young adults up to Filipino adults.

Review on Mental Health Literacy of Filipino Adults

In a series of studies done by Farrer et al., 2008; Fisher & Goldney, 2003; Reavley et al., 2013 they found out that adults in Australia are less accurate than younger adults at identifying symptoms of mental disorders, describe fewer sources of treatment as helpful and are more likely to stigmatize mental illness. In comparison to their American counterparts, Edman and Kameoka's (1997) study found that both educated, and less educated Filipino women ascribe illnesses to spiritual causes: God, chance, witchcraft and sorcery, and spirits. Filipinos were most likely to define the symptoms of relationship troubles, which are thought to be a contributing factor in illnesses. For example, Filipino women in Australia believe that a lack of social support is the leading cause of depression (Thompson, Manderson, Woelz-Stirling, Cahill, & Kelaher, 2002). As a result, people believe that mental health practitioners aren't all that useful because a buddy who can fulfill the same duty and provide basic comfort is readily available' (Thompson et al., 2002, p. 685).

In a classic of Jorm et al. in 1997, they assessed the mental health literacy (MHL) of the Australian population aged 18-74 years old, they interviewed a total of 1010 participants. And their study findings concluded that: Most of the participants recognized the existence of a mental disorder: 72% for the depression scenario (properly diagnosed as depression by 39% of the participants) and 84 % for the schizophrenia vignette (correctly labeled by 27 %). When various people were judged as likely to be useful or detrimental for the person portrayed in the depression vignette, general practitioners (83%) and counselors (74%) were most often rated as helpful, while psychiatrists (51%) and psychologists (49%) were rated as less helpful. Corresponding data for the schizophrenia vignette were: counselors (81%). GPs (74%) are the most common, followed by psychiatrists (71%) and psychologists (62%). Many standard psychiatric treatments (antidepressants, antipsychotics, electroconvulsive therapy, admission to a psychiatric ward) were more often rated as harmful than helpful, and some non-standard treatments were rated highly (increased physical or social activity, relaxation, and stress management, reading about people with similar problems). Vitamins and special diets were more often rated as helpful than were antidepressants and antipsychotics.

Methodology

Research Design

This study utilized a quantitative research design that employed a descriptive online survey method. Quantitative analysis entails generating numerical data or data that can be converted into usable statistics in order to measure a problem. It is used to measure attitudes, beliefs, behaviors, and other well-defined factors, as well as to extrapolate outcomes from a broader group of people (Jerusalem et al., 2017). In reference to the study of Thai et al. (2020) entitled Mental Health Literacy and Help-Seeking Preferences in High School Students in Ho Chi Minh City, Vietnam, in which they utilized quantitative design, describing the categorical data (demographics) using frequency and percentages and quantitative data (scores) using means and standard deviations.

Respondents of the study

Through random sampling, the scale was administered to one hundred thirty respondents (54 males and 76 females) Filipino adults living in Metro Manila, who are aged forty to sixty-five (40-65) years old. They were given informed consent before participating in the study, which was incorporated in the first part of the online survey questionnaire. Other socio-demographic information such as highest educational attainment, marital status, religion, and socio-economic status were gathered for a descriptive analysis of data.

Research Instrument

The researchers used a profile sheet to gather the demographic inventory of the respondents. The researchers also adapted a scale of Connor & Casey (2015): The Mental Health Literacy Scale (MHLS): A new scale-based measure of mental health literacy. This scale consists of thirty-five (35) items that measure the divided components of mental health literacy. It explores the various dimensions of mental health, especially mental disorders, and mental distress. The respondents rated the degree of the trueness of statements that apply to them using a Likert scale. The first ten items of the mental health literacy components, it measures the respondents' ability to recognize specific disorders and distress by presenting a situational statement and classifying diagnostic information. The Likert scale scoring ranges from 1-very unlikely (*siguradong hindi totoo*), which the information and situation presented does not

apply to them; 2-Unlikely (*maaaring hindi totoo*), which means that they seldom do/feel the circumstance and the information applies to them; 3-Likely (*maaring totoo*), which means that they sometimes do/feel the circumstance presented; 4-Very likely (*siguradong totoo*), which means that they do/feel the situation presented on a regular basis. For the succeeding items, 11 to 12, it measures the respondents' knowledge and beliefs about self-help interventions, having the Likert scoring that ranges from 1-very unhelpful (*labis na hindi makatutulong*); 2-unhelpful (*hindi makakatulong*); 3-helpful (*makatutulong*); 4-Very helpful (*labis na makakatulong*). In items 13 to 15, it measures the respondents' knowledge and beliefs about the availability of professional help, having Likert scoring, ranging from 1-Very unlikely (*siguradong hindi totoo*); 2-Unlikely (*maaaring hindi totoo*); 3-likely (*maaring totoo*); 4-Very likely (*siguradong totoo*). Meanwhile, items number sixteen (16) to nineteen (19) measure the respondents' attitude to facilitate recognition and appropriate help-seeking behavior. For items number twenty (20) to twenty-eight (28), it is intended to assess the respondents' knowledge on how to seek mental health information, having Likert scoring, ranges from 1-Strongly disagree (*lubos na hindi sumasang-ayon*); 2-Disagree (*hindi sumasang-ayon*); 3-Agree (*sumasang-ayon*); 4-Strongly disagree (*lubos na sumasang-ayon*). Item number 29 to 35 measures the respondents' knowledge about risk factors and causes of other people with mental illness and distress, having Likert scoring, ranges from 1-Definitely unwilling (*tiyak na hindipumayag*); 2-Probably unwilling (*maaaring hindi pumayag*); 3-Probably willing (*maaring pumayag*); 4-Definitely willing (*malugod na pumayag*).

Data Gathering Procedures

The researchers applied the following procedures in order to collect data:

- (1) Creating an online google docs form and posting it to various social media platforms.
- (2) Signing of the consent form was provided indicating the respondents' voluntary participation in the study.
- (3) Online administration of the scale via google forms and collecting the data;
- (4) Analyzing the results using statistical treatment in light of the questions of the study.
- (5) Formulating the conclusions and recommendations.
- (6) Proposal of mental health literacy programs based on the results of the study.

Results and Discussion

This section presents, analyzes, and interprets the data obtained during the study. The presentation of data includes the analysis and differences amongst the six component variables of the Mental Health Literacy (MHL) under study.

The General Level of Mental Health Literacy (MHL) Among Filipino Adults

Table 1. *The Average Score of the Respondents According to Mental Illness Recognition*

<i>The Six Mental Health Literacy (MHL) Components</i>	<i>Mean</i>	<i>Verbal Interpretation</i>
Mental illness recognition	3.11	Moderate literacy
Knowledge of self-help intervention	3.15	Moderate literacy
Knowledge and beliefs about support available	2.96	Moderate literacy
Attitudes of recognition and help seeking behavior	3.12	Moderate literacy
Knowledge about risk factors and causes	2.24	Marginal literacy
Knowledge of mental health influences	2.45	Marginal literacy
Mean Average of MHL	2.74	Moderate literacy

Table 1 shows the total average mean score of Filipino adult respondents in every six components of mental health literacy. It is noticeable that among these six components, the knowledge of mental health influences and knowledge about risk factors and causes only got a marginal literacy level. This insinuates the need to focus on the proposed educational program to further improve these two components commonly associated with social and personal stigma among Filipino adults towards one's mental health condition. Furthermore, mental illness recognition, knowledge of self-help intervention, knowledge, and beliefs about the support available, and attitudes of recognition and help-seeking behavior are all found in a moderate literacy level. These results suggest that they have knowledge on the areas mentioned above of mental health literacy, but they are uncertain, which indicates the need for further enhancement of this awareness.

The Significant Difference Between the Demographic Profile of the Respondents and the Six Mental Health Literacy (MHL)

Table 2. *The difference of mental illness recognition of Filipino adults when group according to their demographic profile*

<i>Mental Illness recognition</i>	<i>F-value</i>	<i>P-value</i>
Socio-Economic Status	1.81	0.46
Religion	0.65	0.561
Marital Status	2.397	0.359
Highest Educational Attainment	0.99	0.015

Based on this table, the computed P-values are 0.46, 0.561, 0.359, and 0.015 for all the demographic profiles presented. The highest Educational Attainment is the only variable with a computed P-value of 0.015 $\leq$ 0.05, therefore the researchers rejected the null hypothesis of no significant difference of Mental illness recognition on this variable. All the other remaining demographic variables greater than the confidence interval of 0.05 are accepted with the possibility of no significant difference of mental illness recognition on socioeconomic status, religion, and marital status.

The results based on this component confirm that educational attainment influences the Mental Illness recognition of Filipino adults. Aligned to the study of Dela Crus's et al., in 2020 published under the University of the East-Ramon Magsaysay under the Health Sciences Journal, they stated that among the Filipino residents of Barangay Tibagan in San Juan City, they conclude that Filipino respondents ages eighteen to sixty-five (18-65) were generally knowledgeable about mental health illness, and they further added that there was a general acceptance and less stigmatizing attitude in the population. Furthermore, they concluded based on their study, most of the Filipino residents had an awareness that they were relatively high in recognizing depression, stress, bipolar disorder, and drug addiction as mental illnesses by most respondents. In a comparable study about Mental Health Literacy (MHL) conducted by Ines (2019) at the De La Salle University, she concluded that when it comes to the Filipino college students, they were able to recognize symptoms of depression in a given vignette using the correct label and 95.7% with an intention to seek help wherein this could be because individuals with higher education are generally more exposed and well informed about mental health than its counterparts. A national survey in Australia reported that in general educational attainment can have an impact on the mental health literacy level of a given population, while having tertiary education was significantly associated with higher mental health literacy scores, particularly on depression, chronic schizophrenia, and post-traumatic stress disorder (Reavley, Morgan, and Jorm., 2014). Adding to this, the institutional bodies that manage public education and awareness campaigns on mental

health and mental disorders in the Philippines have something to do with this since these institutions are crucial in shaping the general public's mental health literacy or awareness. These bodies include government agencies, NGOs, professional associations, media, and private foundations that have promoted public education and awareness campaigns, often targeting the general population.

Consequently, no significance was found between socioeconomic status and mental health recognition. Based on the systematic review of Martinez's et al. in mental health problems and associated barriers and facilitators; they highlighted that financial constraints that fall in economic barriers are more evident in impeding a person's ability to access mental health services and support rather than in recognition of mental health problems, which is according to them can be more associated on the psycho-social barrier of a person in having inadequate awareness of knowledge on perceiving psychological distress. Likewise, there was also no significant difference found in religion, this is contrary to the report conclusions of Tan (2008), that Filipinos recognize that psychological illnesses are attributed to the religious beliefs as retribution from God and other spiritual causes such as spirits, possession that are primarily folk conceptualization of mental health illnesses. Additionally, marital status has also been found out to have no significant difference in recognizing mental illness. This is congruent to the reported findings of Hartini's et al., 2018, that there were no significant score differences in recognition attitude of their respondents on mental health illness between single, married, divorced, or in between respondents in this specific domain, primarily due to the marriage couples view on mental health conditions or illness of a person as something deviant in the norm that made someone somehow unique and different; thus their attitude become progressively accepting and tolerant on these differences instead of correctly labeling it.

Table 3. *The difference of knowledge of self-help intervention of Filipino adults when group according to their demographic profile*

<i>Knowledge of self-help intervention</i>	<i>F-value</i>	<i>P-value</i>
Socio-Economic Status	2.736	0.044
Religion	2.265	0.341
Marital Status	1.063	0.43
Highest Educational Attainment	3.755	1.42

Table 3 shows the computed P-values of 0.044, 0.341,

0.43, and 1.42 for the demographic profile presented. Only Socio-Economic Status has a lower P-computed value of $0.044 \leq 0.05$, therefore the researchers rejected the null hypothesis of no significant difference of knowledge of self-help prevention to socio-economic status. On the other hand, all other demographic profiles calculated a greater P-value than the confidence interval of 0.05 hence; the researchers accepted the possibility of no significant difference of knowledge of self-help intervention on religion, marital Status, Highest Educational Attainment.

From an economic and cultural standpoint, the majority of mental health experts and institutions are concentrated in cities and the capital, Manila, with a professional-to-population ratio much below the minimum requirements. Counselor and psychologist professionalization is similar in its early phases. This increases Filipinos' dependence on traditional and folk healers, who are easier to find and less expensive than mental health experts (Tuliao, 2014). In addition, Filipinos' thinking, and behavior are still influenced by traditional Asian attitudes, beliefs, and rituals. Furthermore, 42% of the psychotic patients had sought help from traditional healers before seeking psychiatric help (Lin, Demonteverde, & Nuccio, 1990). According to the study of Behere, P. B. (2013), religion has not always shown an association between religious groups and the mental health of adults. Respondents have their own way of coping and seeking help about mental health concerns, and it does not involve the person's religious affiliation. They tend to cope up on their own, and they set aside any other factors. Furthermore, for marital status, it is shown that it is not a significant predictor of the self-help-seeking behavior of the respondents. Not all relationships are positive and marital disruption like divorce or separation can cause emotional distress that will block the situation from self-seeking behavior. Individuals who experience this can suffer from stress and other mental health conditions due to transition. Consequently, some factors are changes in support or relationships, demands, change of residence, and financial support (Cotten, S.R., 1999).

Table 4. *The difference of knowledge and beliefs about the support available of Filipino adults when group according to their demographic profile*

<i>Knowledge and beliefs about support available</i>	<i>F-value</i>	<i>P-value</i>
Socio-Economic Status	2.261	0.008
Religion	0.905	0.561
Marital Status	1.106	0.359
Highest Educational Attainment	2.087	0.015

The computed P-value for the presented demographic profiles above is 0.008, 0.561, 0.359, and 0.015. Socioeconomic status and Highest Educational Attainment P-values are 0.008 and 0.015 less than equal on the confidence interval of 0.05, hence, the researchers rejected the null hypothesis of no significant difference of knowledge and beliefs about the support available on these two demographic profiles. Religion and Marital Status calculated a higher P-value of 0.561 and 0.359 compared to a confidence interval of 0.05, the researchers accept the possibility of no significant difference for these two presented variables.

This indicates that the individual's socioeconomic status matters in having the awareness and beliefs about the support available by a mental health professional. This result corresponds with the study of Amirah et al. (2020), they postulated that family income is significantly associated with awareness of the support availability of mental health professionals that contributes to one's mental health literacy (MHL). It was concluded in their study that the majority of the Malay group and Muslims were motivated towards using traditional or complementary medicines and treatments for mental conditions due to the cost of proper mental health treatments and services. Moreover, based on the systematic review of Martinez, A.B. Co. Lau, M.J., Brown, J.S. (2020) entitled "Filipino help-seeking for mental health problems and associated barriers and facilitators." They emphasize the practical barriers to the use of mental health services like accessibility and financial constraints are also consistently rated as significant barriers in Filipinos encounter being aware of the support available provided by mental health professionals. They further discussed that Filipinos are confronted with social problems of lack of mental health facilities, services, and professionals due to meager government spending on health. Accordingly, in the same study of Martinez et al. (2020), they crossed reference the study done by Saxena et al. in 2007. They stated that in the Philippines, mental health services are too costly and inaccessible; thus, financial constraints hinder help-seeking and become a factor in being aware of the support provided. In line with this, in a qualitative study of Straiton's et al. (2018) entitled: "It has not occurred to me to see a doctor qualitative of feeling," one of their research participants mentioned, "In the Philippines...[Filipinos] It takes a really long time for us to realize that this condition is serious. We [Filipinos] don't want to use our money right away" [p.6]. While there was no evidence found specifically

on the significance of the highest educational attainment to knowledge and beliefs about the support available for people with mental illness, literacy, in general, enables understanding of prescription and medical treatment instructions, as well as obtaining and comprehending provider and public health messages about health-promoting behaviors, health-care check-ups, and screenings, all of which can influence participation in health-promoting behaviors and health status (Nguyen et al., 2017). In contrast, in the study of Lee, Hwang, Ball, Lee, & Albright, 2020, Higher MHL was associated with at least a bachelor's degree in higher education. The most widespread assumption in the available literature is that educational outcomes have a significant positive association with MHL. Higher educated people have more choices, so they gain greater control and peace of mind and security. Graduates are more likely to earn a lifetime. Lower education levels, on the other hand, are associated with "lack of psychological resources" (such as control, resilience, satisfaction and the ability to postpone access to cultural activities) and increased exposure to daily stressors (Neimeyer et al. 2020; Williams, 2021). Low MHL was associated with lack of education and financial problems. These findings may be due to several factors. Financially disadvantaged people have limited access and are less likely to use mental health services for financial reasons and are therefore less aware of mental health issues. In addition, their limited financial resources can prevent them from participating in mental health improvement activities (Schneider et al. 2021).

Furthermore, it was also found that there was no significant difference between marital status and knowledge and beliefs about the support available. This is primarily due to the gender roles differences of husband and wife in perceiving the need for support of mental health professionals differently to which further posits that the role of family characteristics in mental health awareness in acknowledging support in the shared marital context was complex (Grundy & Read, 2011). Moreover, this can also be attributed to the composition of age differences of the respondents' focus on their matrimonial profile and cultural factors that may explain these differences in the result. Similarly, it was also found that religion has no significance to knowledge and beliefs about the support available. In the study of Jang et al. 2016, religious leaders identified religious practices, counseling, psychotherapy, hospitalization, and medicine as treatment options but put a stronger emphasis on religious practices such as saying, attending church services, counseling with clergy, or reading bibles.

Table 5. *The difference of attitudes of recognition and help-seeking behavior of Filipino adults when group according to their demographic profile*

<i>Attitudes of recognition and help seeking behavior</i>	<i>F-value</i>	<i>P-value</i>
Socio-Economic Status	2.179	0.007
Religion	0.932	0.542
Marital Status	1.089	0.373
Highest Educational Attainment	1.115	0.347

On this table, the computed P-values are 0.007, 0.542, 0.373, and 0.347. Socioeconomic status has a computed P-value of $0.007 \leq 0.05$. Therefore, the null hypothesis of no significant difference between socioeconomic level and attitudes of recognition and help-seeking behavior was rejected.

Help-seeking for mental disorders is linked to attitudes about the efficacy and usefulness of treatments and therapies. Recognizing a problem is usually seen to be the first step toward getting help. In the report of Romualdez (2021), in the first five months of 2021, Indonesia, the Philippines, and Malaysia saw the greatest increase in Google searches for mental health services when compared to the same period in 2019. However, Southeast Asians, especially the Philippines, have turned to do-it-yourself solutions because mental health care can be costly and even taboo for some. Additionally, Pring and Magnaye (2012) assess caregivers of patients with maladaptive behavior awareness and use of mental health treatments in Batangas Province. It was discovered that caregivers were aware of the Department of Health's many health services; however, not all of them were provided in the medical facility where they sought advice. The literature reviews stated above were supported by the study of Martinez et al. (2020), stating that financial restrictions and inaccessibility of services are among the barriers highlighted by Filipinos living in the Philippines when it comes to seeking assistance. This is due to the fact that families with a low socioeconomic level (SES) are deprived in numerous ways and face a greater number of stressors connected to finances, social relations, employment situations, and health concerns than families with a high SES. On the subject of religion, this study found out that there is no significant difference between the attitude of recognition and help-seeking behavior, and religion which contradicts the study of Ward et al. (2013), which found that the religion of African Americans impacted their mental health help-seeking behaviors.

Moreover, in the study of Prabhu et al. (2021) he

conducted a study about the mental health literacy of school teachers whose age was between 26 to 46 and above years old. Their study found out that the respondents' marital status and self-efficacy in connection to seeking mental health information, as well as their reported ability to raise awareness and provide referrals, were significant predictors of mental health problems among their wards. However, higher levels of religiosity did not lead to a decrease in the number of people seeking help from mental health specialists. Religious people seemed more at ease getting help from clergy, while this did not appear to impact their behavior when seeking treatment from mental health specialists (Abe-Kim, Gong, & Takeuchi, 2004). Correspondingly, no significant difference was found in marital status. This contradicts the study of Adewuya and Makanjuola (2008), in which they stated that clinical diagnosis, religious affiliation, marital status, and educational qualification were found to predict mental health help-seeking behaviors. While in the highest educational attainment, the researchers found no significant differences between the two. The result was supported by the study examined by Nnabuchionye (2020) about the association between educational attainment and the intention to seek professional psychiatric help. In addition, findings from statistical analysis found no significant difference in the attitude of Nigerian immigrants regardless of educational level.

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Table 6. *The difference knowledge about risk factors and causes of Filipino adults when group according to their demographic profile*

<i>Knowledge about risk factors and causes</i>	<i>F-value</i>	<i>P-value</i>
Socio-Economic Status	2.313	0.006
Religion	1.635	0.71
Marital Status	1.791	0.041
Highest Educational Attainment	0.582	0.892

Table 6 presented the socio-economic status and marital status tally a P-value of 0.006 and 0.041, respectively. Based on the criteria judgment, the computed P-values are less than equal to the

confidence interval of 0.05, and the researchers rejected the null hypothesis of no significant difference of knowledge about risk factors and causes on socio-economic status and marital status. Meanwhile, religion and highest educational attainment computed a larger P-value compared to 0.05. Thus, the researchers accept the possibility of no significant difference for these variables.

Individuals with higher income reported a greater desire for social distance from people with substance abuse disorders (Alexander and Link, 2003); these findings are consistent with recent research by Goodyear and Chavanne (2020), which found that people with lower income tended to have lower levels of stigma toward people with opioid addiction. While these findings were demonstrated in relation to substance abuse disorders, which are frequently viewed differently than other mental illnesses, links between higher income and various mental illness stigma outcomes have also been demonstrated in relation to people with schizophrenia, depression, and nondescript mental illness (Anglin et al., 2006; Givens et al., 2007; Hartini et al., 2018; Venkatesh et al., 2015). In a vignette character with schizophrenia, Holman (2015) discovered that higher SES was connected with higher levels of stigma, which is consistent with earlier research indicating a positive relationship between income and mental illness stigma (Alexander & Link, 2003; Villatoro et al., 2018; Martin et al., 2000). Finally, Foster and colleagues (2018) discovered that, due to decreased empathy and mental illness knowledge, higher SES was associated with higher levels of nondescript mental illness stigma and clinical depression stigma, despite the fact that this finding was based on subjective rather than objective SES measures. Although mental health literacy has never been thoroughly investigated in terms of the difference in marital status, one study by Sayed et al., 2021 found that marriage is seen to be an experience that enables a person to embrace the diversity and uniqueness of others gradually. This is especially true for married couples who live together. As a result, married people are more understanding of persons who suffer from mental illnesses than single or divorced people. Moreover, the highest educational attainment was found to be insignificant to knowledge about risk factors and causes. This is in line with the study of Hartini et al., 2018 wherein the difference in levels of stigma toward mental health patients was shown to be unrelated to education level. This conclusion implies that a person's educational background has little bearing on public perceptions of people with mental illnesses. Similarly, it was also found that religion is not a significant factor which accord to the study of

Bushong (2018), indicated that levels of religiosity across faith communities have no significance to levels of mental illness stigma.

Table 7. *The difference Knowledge of Mental health influences of Filipino adults when group according to their demographic profile*

<i>Knowledge of Mental health influences</i>	<i>F-value</i>	<i>P-value</i>
Socio-Economic Status	1.541	0.102
Religion	1.147	0.324
Marital Status	1.897	0.03
Highest Educational Attainment	2.239	0.009

Marital Status and Highest Educational attainment tally a P-value of 0.03 and 0.009, respectively. These two variables are less than the confidence interval of 0.05, and thus, the researchers can reject the null hypothesis of no significant difference. Socio-economic and Religion presented a higher P-value of 0.102 and 0.324 contrasted to the 0.05 confidence interval.

For these variables, the researchers accept the hypothesis for the possibility of no significant difference. For these, we will not reject the hypothesis for the possibility of no significant difference. In a series of reviews of fifteen years' worth of public belief/attitude stigma research, Angermeyer and Dietrich (2006) stated that those with higher levels of education desired less distance from the 'mentally ill' and expressed more liberal views. Furthermore, previous research findings support the results that lack of knowledge on mental health or mental health literacy is one of the sources of stigma wherein they tend to discriminate against the people who suffered mental health problems (Hartini, N. et al., 2018). On the other hand, in research of Hartini, N. et al. (2018), it was discussed there that marriage life, particularly couples who lived together, is considered an experience that allows an individual to embrace the differences and uniqueness of other people gradually. Married people are shown to be more open and tolerant toward people with mental disorders if we are going to compare them to single individuals. Moreover, one study discussed that each person in the marriage life could bring each political participation history to the marriage. But that exchange of participation can be borne out to a shared set of social and economic circumstances as well as opportunities to learn from and be influenced by each other (Stoker, 1989). It makes sense for couples to share political influence and other aspects of life during their

marriage journey.

When it comes to socioeconomic status, it is believed that regardless of people's status, people tend to be imposed discriminatory attitudes towards people who have a mental illness, and one of the major reasons behind that is for the safety of both the individual and the community. For the aspect of Religion, it resulted that the Religion of an individual does not influence one's view when it comes to the discriminatory attitude and stigma of people who suffered mental health. They still tend to ignore that people who suffer from mental health are dangerous, wherein they usually have difficulties in understanding and empathizing with their behavior and situation. Religious affiliation has been shown to have no impact on mental health literacy or attitudes towards people with mental illness. On the other hand, a higher level of religion was associated with a negative attitude towards the mentally ill, but this relationship became less important given the demographic characteristics (Wang et al., 2019).

Conclusion

Based on the generated general mental health literacy (MHL) result, it was found that Filipino adults residing in Metro Manila have a moderate literacy level, which means they have awareness about mental health; however, they are uncertain about it. This indicates that the knowledge that they have is limited. Among the six components, four fall in a moderate literacy level, namely, knowledge of self-help intervention, attitudes of recognition and help-seeking behavior, mental illness recognition, and knowledge and beliefs about the support available. However, two of the components show a notable result found in the marginal literacy level, which are the knowledge about risk factors and causes and knowledge of mental health influences. In terms of the significant difference in the demographic profile of the Filipino adults, it was found that socioeconomic status is a significant factor in the knowledge of the self-help intervention, knowledge, and beliefs about the support available, attitudes of recognition and help-seeking behavior, and knowledge about risk factors and causes. Marital status, on the other hand, was found to be significant to knowledge about risk factors and causes and knowledge of mental health influences. Finally, the highest educational attainment was found to be significant to mental illness recognition, knowledge, and beliefs about the support available, and knowledge of mental health influences.

In view of the previously mentioned results, the following recommendations were offered:

1. Future researchers are encouraged to use a mixed-method design in order to have a more comprehensive and in-depth discussion about the levels of Mental Health Literacy of Filipino adults.
2. Future researchers are encouraged to create a localized standardized test about Mental Health Literacy (MHL).
3. The researchers recommend carrying out a comparative study on the Mental Health Literacy of Filipino adults within Rural and Urban Areas in the Philippines.
4. Future researchers are encouraged to discover more about the concept of personality factors influencing the levels of Mental Health Literacy of Filipino adults.
5. Future studies pursuing similar goals may improve the findings by extending the scope of this study to other areas in the pursuit of a mentally literate society.
6. It is important to replicate the current study with more diverse samples, in regard to occupation, age, and other socio-demographic variables. It may also be interesting to examine whether different components of mental health literacy are important in different sub-groups of the community.
7. The researchers also recommend implementing the proposed program in this study to mental health organizations and institutions.

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